جوستاف روسي انترناشيونال - مصر (EPE) * **طلب علاج أول مرة / متردد**

إسم المريض رباعياً : ايمان عبيد محمد محمود
المحافظة : الجيزه **الحي / المدينة :** اخري **العنوان :** بولاق الدكرور
رقم البطاقة : 27111282101767 **رقم التليفون :** 01128085977 **تاريخ تسجيل الإستمارة :** 21-10-2026
العمر(تاريخ الميلاد) : 28-11-1971 **المهنة :** ربه منزل

الإقرار

أقر انا الموقع أدناه انني لا اتمتع باي نظام علاج مثل:-التأمين الصحي-القوات المسلحه-الشرطة-الهيئات-المؤسسات او اي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة نفقات العلاج في حالة ثبوت غير ذلك

الإخصائي الاجتماعي **المقر بما فيه**
التوقيع:(----------) **التوقيع:**(----------)

التشخيص الإكلينيكي : سرطان بالثدي (مبادرة)
نتائج الأبحاث : (مرفق)
التشخيص النهائي : سرطان بالثدي (مبادرة)
خطة العلاج : علاج مبادرة سرطان الثدي علاج تدعيمي وتلطيفي [6579] **قسم العلاج :** الاورام
التعليق على التشخيص : ----------

تقرير اللجنة الثلاثية المتخصصة

الاسم: د/محمد كيلاني **الوظيفة:** ---------- **التوقيع:** ----------
الاسم: د/ندى حسن **الوظيفة:** ---------- **التوقيع:** ----------
الاسم: د/نرمين رمضان **الوظيفة:** ---------- **التوقيع:** ----------

أطلعت اللجنة علي لائحة العلاج الخاصة بالمجالس وتلتزم جهة العلاج بلائحة المجالس الطبية.
كما تقر اللجنة بأن البيانات المدونة فى هذا النموذج صحيحة.

تعتمد صحة البيانات المرفقة
رئيس اللجنة و مدير المستشفي.

تتعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناظرة المريض بالمستشفي قبل الصرف.

أعضاء اللجنة

ا.د/ ---------- **ا.د/** ---------- **د/** ---------- **د/** ----------

قرار اللجنة

التشخيص النهائي : ----------
خطة العلاج : ----------
المستشفي : ----------**قسم :** ----------
جهة العلاج : ----------**نفقات :** ----------
ملاحظات : ----------

رئيس	مدير عام	ختم
المجلس الطبي المتخصص	مديرية الشئون الصحية	شعار الجمهورية

Name: ايمان عبيد محمد محمود
National ID: 27111282101767

تقرير طبي

السيد

تحية طيبة وبعد,,

إيماء للطلب المقدم من	**بشأن تقرير طبي للمريض**
بقسم:	**رقم طبي:**

نتشرف بأن نوضح فيما يلي تقريرا عن حالته

تاريخ وساعة الدخول : -- --:-- yyyy-mm-dd	**تاريخ وساعة الخروج :** PM 01:17 05-07-2026

وبعد الكشف والفحوصات والمتابعة وجد انة

المريضة تعاني من ورم غير حميد بالثدى و تحتاج الي Supportive and palliative Treatment

التاريخ : 05-07-2026	**الوقت :** -- --:--
	الطبيب المعالج : د/شيماء لاشين





National ID · Front & Back
[[DOCTYPE:ID_FRONT_BACK]]

Patient Name	: الأستاذه / ايمان عبيد محمد محمود	Request No.	: 13001404
Patient No.	: 27111282101767	Request Date	: 08/07/2024
Path No.	: 2302/24	Verification Date	: 27/07/2024
Sex	: Female	Financial Category	: اورام دار السلام - نفقة الدولة
Age	: 52 Y	Department	: Out Patient
Referreing Doc.	:	IHC No.	:
		Registered by	: Mrs.Mayada Hegazy

IMMUNOHISTOCHEMISTRY REPORT

Clinical Data: Known case of breast cancer with right hepatic focal lesions.
Nature of Specimen: Tru-cut biopsy, collected at 7/7/2024.

Technique:
- Sections were prepared from the paraffin block, processed in Bench Mark ULTRA machine (Ventana) and stained by rabbit monoclonal antibodies using ultra view HRP DAB as chromogen and hematoxylin as counter stain. The sections treated against:
- ER clone (1E2), PR clone (SP1), Her2 clone (4B5), & Ki67 clone (30-9).

Results:
* IPT (Positive control showed appropriate reactivity):
- GATA3: Diffuse strong positive.
- ER: Strong nuclear reaction in 90% of tumor cells (score 8/8).
- PR: Faint nuclear reaction in 3% of tumor cells (score 3/8).
- Her2: Faint incomplete membrane reaction in >10% of tumor cells (score 1+).
- Ki67: 10% nuclear reaction of tumor cells.

Conclusion:
Known case of breast cancer with right hepatic focal lesions, Tru-cut biopsy:
Metastatic adenocarcinoma of mammary origin.
IPT:
- GATA3: Diffuse strong positive.
- ER: Positive (score 8/8).
- PR: Positive (score 3/8).
- Her2: Score 1+.
- Ki67: Positive in 10% of tumor cells.

Verified by :
Dr.Toka Medhat

غرفة رقم (4) – الدور الأول – مبنى رقم (987) – كورنيش النيل – مصر القديمة – القاهرة – مصر
Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadema - Cairo, Egypt

Immunohistochemistry
[[DOCTYPE:IHC]]

Name: ايمان عبيد محمد **MR. :** 894	

Detailed Medical Report

السيد

تحية طيبة وبعد,,

إيماء للطلب المقدم من		**بشأن تقرير طبي للمريض**	
بقسم:		**رقم طبي:**	

ننشرف بأن نوضح فيما يلي تقريرا عن حالته

تاريخ وساعة الدخول :		**تاريخ وساعة الخروج :**	

وبعد الكشف والفحوصات والمتابعة وجد انه

Gemzar/Carboplatin + Zometa Q 3 months CT CAP (11/2025): RD For C5D8 Gemzar 1600mg.

55 Y WT 61 HT 155 POSTM MBC TO LIVER LUNG BONE (STERNUM,) HFL BIOPSY ER8/8 PR3/8 HER2-VE RT BCS 15 Y AGO METS IN 2022 ST FASOLDEX KISQALI DP AFINITOR AROMASIN DP XELODA X2 SHIFTED TO NAVELBEINE DP REC TAXOL DP REC 3 CYCLES GEMZAR CARBOPLATIN CT RD REC UP TO C5 D8 GEMZAR ONLY PT IS NEUTROPENIC TO POSTPONE 1 WEEK.

for c6 gemzar 1600 carbopl auc 5 zometa .c6 d8 gemzar only pharmacy due date 25/2.

FOR C7 D1 GEMZAR 1600 + CARBO AUC 5 PHARMACY + GEMZAR ONLY D8 + neupogen LABS acc.

MBC 3/2026 CT CAP :SD TODAY : C 8D8 PORTA CATH PENDING.

cts 25/3/2026 sd of rt pleural eff sd of med ln sd of bilobar hfl for c9 d1 gemzar carboplatin auc 5 d8 ph.

Pharmacy Visit FOr C9D8 Gemzar 1600 Labs 22/4 ... accepted.

anemia 8.8, anc 1.52 c10d1 gem, carbo auc 5 with neupogen d8 pharmacy with cbc.

Pharmacy Visit For C10D8 Gemzar 1600 Labs 20/5 ... hgb 8.5 acce3pted.

cts on 25/3/2026 sd encysted rt pleural eff sd of no size of bilobar hfl largest m 2.9x1.8 cm for c11 d1 gemzar 1600 carboplatin auc 5 d8 gemzar only ph for new cts.

Pharmacy Visit For C11 D8 Gemzar 1600 Labs 17/6 .. accepted.

for c12 gemzar carboplatin d1, d8 pharmacy Hb 7.5 > PRBCs transfusion, 1 proph neupogen skin fungating soft tissue lesion on the chest, CTs pending.

ct cap done without comparison ???? for comparison with viewing films marked RT encycted pleural effusion (non sympotomatic) for IR consultation regarding tapping still present chest wall mass (SD) meanwhile contiune her systemic ttt for c12 D8 after labs + zometa pancytopnia to omit cycle this week and consider carbo auc 2 d1& d8.

تحريرا في ,

التاريخ :	**20-09-2026**	**الوقت :**	

الطبيب المعالج : **dina makram**

يعتمد ,

cts 18/7/2026 sd ofmarked rt pleural effusion sd of pretracheal ln ,sd of bilobar hfl largest seg 4 m 29x18 sd of sclerotic foci on exam sternal cutaneous ulcerative lesion 2x2 (for fu if progressing for mdt) as pt hads delayed multiple times bec of anemia threombocytopenia for 10% dr for c12 d1 gemzar 1400 carbopl auc 2.

bec tlc 2.6+/1.4 for giving chemo and 1 proph neupogen and blood transf and repeat cbc next week for c12 d1 gem carb.

for c12 gemzar 1400 carbopl auc 2 and 1 proph neupogen still ulcerative lesion in chest 2 cm.

plt 35 inspite of dr to postpone 1 week pt did tapping of rt pleural eff sd of ulcerative lesion 2 cm if still thrombocytopenic for mdt to shift line of tt.

for c13 D1 gemzar 1400 carbopl auc 2 and 1 proph neupogen C13 D8 IN PH.

pt has vaginal bleeding for new labs tvus tlc 2/1 hb 7.7 plt 100 on exam still ulcerative sternal lesion for blood transf and postpone 1 week.

The patient has completed 13 cycles of gemzar/carboplatin regimen to date.

Patient Name	: الأستاذه / ايمان عبيد محمد محمود	Request No.	: 13001404
Patient No.	: 27111282101767	Request Date	: 08/07/2024
Path No.	: 2302/24	Verification Date	: 16/07/2024
Sex	: Female	Financial Category	: اورام دار السلام - نفقة الدولة
Age	: 52 Y	Department	: Out Patient
Referreing Doc	:	Registered by	: Mrs.Mayada Hegazy

Pathology Report

Clinical Data: Known case of breast cancer with right hepatic focal lesions.

Nature of Specimen: Tru-cut biopsy, collected at 7/7/2024.

Gross Examination:
- Multiple grayish white tissue cores up to 1cm, totally submitted.

Microscopic Examination:
*Revealed hepatic tissue core showed infiltration by malignant growth formed of malignant glands lined by hyperchromatic pleomorphic cells.

Diagnosis:
Known case of breast cancer with right hepatic focal lesions, Tru-cut biopsy:
Metastatic adenocarcinoma.
For further IHC (GATA3).

Signature
Prof. Dr. Lubna Omar.
Dr. Esraa Magdy.

Verified by:
DR.Menna Nabil

CAP ACCREDITED
غرفة رقم (4) – الدور الأول – مبنى رقم (987) – كورنيش النيل – مصر القديمة – القاهرة – مصر
Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadema - Cairo, Egypt

Pathology Report
[[DOCTYPE:PATHOLOGY_REPORT]]

EBEED MOHAMED, AYMAAN Patient ID: 104005853 DOB: 01-Jan-1971 Date: 15-Jul-2025

Patient Lastname: EBEED Mohamed
Patient Firstname: AYMAAN
HIS Patient ID: 104005853
RIS Patient ID: 116905
Date of Birth: 01/01/1971

Procedure: 70030, Bone Scan
Medical Question:
Clinical Comment:

========== REPORT TEXT ==========

99mTc MDP Bone Scan Report (15/07/2025)
* Whole Body Anterior and Posterior Planar Views Acquired 2Hrs. Post I.V Injection of 20mCi of 99mTc MDP
Revealed:

" Active tracer uptake is seen at the sternum; keeping with metastatic osseous lesion for CT cuts correlation.
" Otherwise, no definite evidence of active osseous deposits could be seen over the rest of the surveyed skeleton.
" Degenerative changes are noted over the dorso-lumbar vertebrae.
" Arthritic changes are noted over both shoulder and both knee joints.
" Both kidneys are visualized.
Conclusion:
" Solitary active metastatic osseous lesion at the sternum as described.

Much Obliged
Randa Ali, MSc

========== REPORT TEXT ENDE ==========

Bone Scan
[[DOCTYPE:BONE_SCAN]]

- Mild progressive course of right lower deep cervical and supraclavicular lymph nodes
- Regressive course of mediastinal lymph nodes ....for PET/CT
- Stationary multiple lytic bony lesions for correlation of bone scan.
- No denovo lesion
- Stationary rest of the study.

Much Obliged;
Dr.Khaled Moghazy,MD
Prof.of Radiology

Dr. Khaled Moghazy

Dr. Khaled Moghazy

CT Scan
[[DOCTYPE:CT_SCAN]]

GUSTAVE ROUSSY INTERNATIONAL EGYPT

Patient Name: Eman Ebeid Mohamed Mahmoud	Date : 15/11/2025
Exam : CT CAP+C (CT Chest Abd. Pelvis)	Age : 54 Year(s)
Medical ID: 443/2022	Pat. ID: 26687

POST CONTRAST MULTI-SLICE CT SCAN OF THE CHEST, ABDOMEN, AND PELVIS: -

Findings:

Status: right breast cancer with metastatic disease.

As compared to prior study dated 30/7/2025, the current study revels :

I Chest:-

- **Resolution of** left axillary lymph nodes
- **Mild progressive course regarding** size of the previously reported right lower deep cervical and right supraclavicular lymph nodes largest measures about 2.3 x1.3 compared to 2.3x 1.3 cm priorly .
- **Stationary course** regarding right side marked encysted pleural effusion and enhancing pleural reflection with sequent near total right lung collapse.
- **Regressive course** of the multiple enlarged partially amalgamated right para-tracheal, pretracheal,pre and subcarinal mediastinal lymph nodes ,
- Multiple upper nodular calcific densities ,likely granulomatous densities.
- No evidence of left pleural effusion.
- Normal cardiac size and shape with no gross abnormality of the cardiac chambers. No pericardial effusion detected.
- Still noted bulky nodular thyroid gland.

II Abdomen & pelvis:

- **Stationary size** of previously mentioned bilobar hepatic focal lesions, the largest is seen at segment IVa measuring about 2.9 x1.8 cm
- Patent homogenously enhancing portal vein and is main branches as well as its main tributaries.
- No intra-hepatic or extra-hepatic biliary radicles dilatation.
- Normal size & parenchymal texture of the spleen with no focal lesions seen.
- Normal size, parenchymal thickness & density of both kidneys with no stones or back-pressure changes.
- Normal appearance of the pancreas, aorta, I.V.C and both suprarenals.

III The Scanned skeleton shows mild progressive course as regards:

1. **Still noted** heterogenous sclerotic appearance of the sternal heterogenous lytic and sclerotic osseous lesions with **stationary size** of the associated pre-sternal soft tissue component about 1.8 x 1.3 cm.
2. **Stationary course of multiple** Small faintly sclerotic osseous lesions seen involving L3 and S2 vertebra.
3. **Multiple lytic lesions of C7 ,dorsal vertebrae**

Opinion:

follow up of metastatic right breast cancer shows:

- **Stationary marked right sided encysted pleural empyema.**
- **Stationary size of the hepatic focal lesions as described.**

CT Scan
[[DOCTYPE:CT_SCAN]]